

**Statement to UNAIDS PCB57:
Operational Guardrails for HIV–Primary Health Care Integration Anchored in Key Population
Leadership**

As the Programme Coordinating Board [PCB] considers the New Global AIDS Strategy (2026–2031), we welcome its focus on sustainability, integration, equity, and people-centred health systems. In line with the Strategy’s priorities on service integration, community leadership, and health system resilience, it is essential to recognize that HIV–primary health care integration is not merely a technical adjustment but calls for more thought in value addition to the service delivery. For key populations, it is a structural reform that directly determines whether access, safety, and health outcomes are preserved or undermined.

Evidence from the [KP-TNC’s 2025 Guardrails for Integration study](#) demonstrates that integration delivers results only when it is governed by clear safeguards and shaped by the leadership of the communities most affected. This statement draws on evidence and perspectives from the Key Populations Transnational Collaboration (KP-TNC), a regional platform led and managed by Key Population Consortia from 9 countries and brings together more than 200 grassroots, community-led key population organizations across Eastern, Southern, and Western Africa. KP-TNC works with governments through various ministries, CCMs, and global partners to strengthen sustainable, rights-affirming HIV responses, including during donor transition and health system integration. The guardrails outlined here therefore reflect lived country experiences and collective community governance priorities.

Key populations have depended on differentiated, community-led HIV services because public health systems have often had challenges in providing safe, confidential, and non-discriminatory care. Integration offers the hope of strengthening continuity, normalizing access, and giving support to long-term sustainability when it is sequenced and responsive to context. However, when integration is driven primarily by donor transition timelines, fiscal consolidation, or institutional restructuring, it risks displacing effective community services before public systems are ready to absorb them. Such approaches do not improve efficiency; they increase downstream costs by accelerating disengagement from care, loss of viral suppression, and preventable new infections, ultimately undermining national HIV targets.

These risks are heightened in the current context of constrained resources, shrinking civic space, increased surveillance by the anti-gender movement and reduced institutional capacity. As responsibilities shift toward national systems and communities, integration without safeguards transfers risk without transferring authority or financing. The New Global AIDS Strategy must therefore treat integration as a managed reform aligned with its commitments to equity, rights, and differentiated responses, rather than as a default pathway to sustainability.

From a community perspective, person-centred integration means coordinated care that remains accessible, confidential, person-friendly and responsive to lived realities, including criminalization, gender inequality, and intersecting vulnerabilities. It requires health settings where key populations, adolescents, young people in all their diversities and being, and gender-diverse communities can seek services without fear of exposure or mistreatment. Consistent with the evidence, integration may take multiple forms

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including community-led models, hybrid or referral-based arrangements, co-located services, or phased primary health care-led approaches with the appropriate modality determined by context, system readiness, and the safety of key populations. Community-led and hybrid models may remain long-term or permanent where they best protect access and outcomes. Integration must expand choice and continuity across HIV, sexual and reproductive health, mental health, tuberculosis, hepatitis, and harm-reduction services. Where primary health care settings are not demonstrably safe or competent, stand-alone and community-led services remain indispensable components of the integrated system.

Community leadership must therefore be embedded structurally, consistent with the Strategy's emphasis on people-centred governance. Advisory roles alone are insufficient. Key population-led organizations must participate in decision-making on service design, contracting, financing, and monitoring. Community Led Monitoring (CLM) is a proven mechanism for safeguarding quality and equity within integrated systems and should be institutionalized within national health reviews and accountability frameworks, including those linked to UHC and social contracting mechanisms. Country CLM mechanisms are working hence the need to support the work being done under CLM.

To operationalize integration under the Global AIDS Strategy, non-negotiable guardrails must be established as minimum conditions rather than optional principles. Integration must not eliminate or defund key population-specific or community-led services where primary health care settings are unsafe or inaccessible. It must not proceed without mandatory provider training, confidentiality protections, and functional accountability and redress mechanisms. Community-led services must not be absorbed into public systems without predictable, multi-year financing that supports continuity and institutional stability. In criminalized or highly stigmatized settings, integration must be accompanied by concrete legal and human rights protections. Integration should not proceed unless these guardrails are demonstrably met, with readiness verified through community-led monitoring and multi-stakeholder review. Where these conditions are not met, integration should be phased, delayed, or redesigned through a formal corrective process of transition which fully and wholly takes into consideration community feedback and advice.

Financing is central to upholding these guardrails and to supporting country ownership. Sustainable integration requires explicit domestic and external resource allocation to community-led service delivery, governance, and monitoring within integrated models. Inclusion of key population-responsive services in national insurance schemes, medium-term expenditure frameworks, and transition plans reduces political and programmatic risk for governments by protecting service coverage and national outcomes. Integration must not be used to justify the re-labeling, dilution, or withdrawal of existing community financing. Evidence from existing social contracting and community-provider models demonstrates that such approaches are feasible and cost-effective when properly financed and governed. Communities should be at the center of such funding models.

The comparative advantage of the UNAIDS Joint Programme is critical to translating these guardrails into practice. WHO should define and uphold standards for key population-competent integrated care. UNDP should anchor legal and human rights protections, particularly in criminalized contexts. UNFPA and UNICEF should ensure that integration responds to the needs of adolescents, young people, women, and gender-diverse populations. The World Bank should support inclusive financing and social protection mechanisms that advance UHC without exclusion. The UNAIDS Secretariat should provide political leadership, ensure coherence across co-sponsors, and monitor adherence to agreed guardrails. The UN

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Family should support diverse communities during this period of transition and integration of health care service delivery.

Progress must be measured. In line with the Strategy's results framework, integration should be assessed and monitored through disaggregated indicators on access, retention, viral suppression, stigma and discrimination in health settings, and coverage of trained providers for key populations. These guardrails should be operationalized from the first year of the Strategy (2026), with initial reporting to PCB58 and a formal mid-term assessment aligned with the Strategy review cycle, enabling timely corrective action where risks emerge.

We therefore call on the Programme Coordinating Board at PCB57 to decide to formally embed these guardrails within the New Global AIDS Strategy and its implementation guidance; to request the Executive Director of UNAIDS to report on early integration impacts on key populations; and to instruct co-sponsors to align 2026–2027 workplans and budgets accordingly. These guardrails do not prescribe uniform models but define minimum conditions that enable countries to pursue integration pathways consistent with national context, ownership, and readiness.

The success of the New Global AIDS Strategy will be judged not by how services are reorganized, but by whether those most affected remain reached, protected, and virally suppressed. Integration that erodes community systems also erodes national health and HIV outcomes. Sustainability without community leadership is not durable. These guardrails constitute minimum conditions for effective Strategy implementation and are essential to advancing equitable, resilient, and accountable HIV responses.

Statement endorsed by KP Consortia of Zanzibar, South Sudan, Zambia, Nigeria, Lesotho, Ghana, Tanzania, Zimbabwe, Malawi and Kenya

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